



Responding to Allegations of Sexual Assault, Reported Sexual Contact Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	All areas	All Clinical Staff

2. Purpose

AKG has a duty of care and legal obligation to provide health care in an environment that promotes personal safety and minimises risk of inappropriate behaviour, including that of a sexual nature, allegedly perpetrated by patients, staff, students, contractors, volunteers, and visitors.

This procedure is to be read in conjunction with the [AKG Sexual Assault Guideline](#).

This procedure aims to guide staff responses to allegations of or suspected sexual assault within inpatient and outpatient mental health services at AKG.

Important Numbers

Child Protection Unit (CPU) 08 6456 4300

SARC 1800 199 888

Sex Assault Squad (WA Police) 08 9428 1600

3. Escalation Process

In the event of an allegation of sexual assault reported by an individual over the age of 18 years; any report to the WA Police must be the decision of the alleged victim. Where the alleged victim is regarded as unable to make an informed decision, the decision will be made by the appointed guardian or personal support person. Should the person be involuntary under the WA Mental Health Act, consultation with the MH Advocate must also occur. For quick reference please see appendix 2.

3.1 Escalation pathway to an individual's disclosure of sexual assault in the inpatient setting:

1. Once a staff member is aware of any interaction of a sexual nature involving inpatients, staff will immediately report the details of the incident to the Shift Coordinator.
2. The Shift Coordinator will ensure strategies are implemented to ensure the patient's safety is immediately considered. This may include movement of room or ward and or increased visual observations. Refer to the [AKG Safe and Supportive Observations: Procedure](#).

Before referencing this procedural document please ensure that you have the latest version from the [Policy Library](#) information hub page.

3. At the earliest opportunity the Shift Coordinator will advise:

- Associate Nurse Unit Manager (ANUM)
- Nurse Unit Manager (NUM)
- Nurse Coordinator Mental Health
- the Treating Consultant
 - Treating Consultant informs HoD Psychiatry.

If afterhours, the [Responding to Allegations of sexual activity – Inpatient After hours checklist \(appendix 3\)](#) is to be completed and the following advised:

- Duty Medical Officer
 - On Call Consultant
 - After Hours Mental Health CNS
 - After Hours Mental Health CNS decides regarding informing the Senior Representative on Call (SROC)
4. The Head of Department Psychiatry or On Call Consultant will ensure that all patients involved are immediately reviewed by the respective treating Consultant Psychiatrist. The HoD Psychiatry will inform the Service Director.
5. The reviewing Consultant Psychiatrist will provide a briefing note to the HoD Psychiatry as soon as practicable.
6. The HoD Psychiatry will assess the situation and resolve what actions should be taken. This responsibility is delegated to the On Call Consultant after hours who will provide a briefing note to the HoD Psychiatry as soon as practicable.
- Actions may include:
 - Immediate review of patients involved, including the offer to refer the complaint to WA Police and the SARC.
 - Continuation of the investigation.
 - The treating/reviewing Consultant Psychiatrist and/or HoD Psychiatry will immediately consult with the WA Police Child Abuse Unit when the individual is under 18 years of age, to determine the required action to be taken.
 - The option of transferring the patient to another service or inpatient area and discussing the option with the patient, guardian and/or Next of Kin.

The [Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#) is to be complied with regarding all reports of alleged sexual assault and/or sexual contact. Datix CIMS is to be used for this purpose. It is expected that the reviewing Consultant Psychiatrist will at minimum provide 3rd party comment or complete the CIMS investigation.

3.2 Escalation pathway to an individual's disclosure of sexual assault in the outpatient setting:

1. Once a staff member is aware of any interaction of a sexual nature involving inpatients, staff will immediately report the details of the incident to the Care Coordinator.
2. The Care Coordinator will ensure strategies are implemented to ensure the patient's safety is immediately considered.
3. At the earliest opportunity the Care Coordinator will advise:
 - Clinical Nurse Specialist (CNS), Senior Social Worker (SSW) or Team Leader (TL).
 - the Treating Consultant
 - Program Manager Community Mental Health
 - The Program Manager then informs the HOD Psychiatry and the Service Director

If afterhours the following are to be advised:

- Duty Medical Officer
 - On Call Consultant
 - Senior Clinician on duty i.e., Clinical Nurse Specialist (CNS), Senior Social Worker (SSW)
 - Senior Clinician makes a decision regarding informing the Senior Representative on Call (SROC)
4. The Head of Department Psychiatry or On Call Consultant will ensure that all patients involved are offered review as soon as practicable or access appropriate services externally, i.e., Emergency Department, Police or SARC.
 5. The reviewing Consultant Psychiatrist will provide a briefing note to the HoD Psychiatry as soon as practicable.
 6. The HoD Psychiatry will assess the situation and resolve what actions should be taken. This responsibility is delegated to the On Call Consultant after hours who will provide a briefing note to the HoD Psychiatry as soon as practicable.
 - Actions may include:
 - Immediate review of patients involved, including the offer to refer the complaint to WA Police and the SARC.
 - Continuation of the investigation.
 - The treating/reviewing Consultant Psychiatrist and/or HoD Psychiatry will immediately consult with the WA Police Child Abuse Unit when the individual is under 18 years of age, to determine the required action to be taken.
 - The option of transferring the patient to another service or inpatient area and discussing the option with the patient, guardian and/or Next of Kin.

4. Considerations for a clinician's response to an individual's disclosure of sexual assault:

The below information is to guide the treating Registrar or Psychiatric DMO in the management of a disclosure. Please refer to appendix 1.

If a sexual assault is disclosed by a patient the staff member to whom disclosure is made is to document disclosure at earliest opportunity and be prepared to provide statement to Police if necessary.

4.1 For all incidents of disclosure

1. Acknowledge disclosure and provide immediate medical care as a priority where necessary – follow ABCDE protocol (Airway, Breathing, Circulation, Disability, Exposure)
 - Call an ambulance/assist patient to the nearest ED if not at AKG or arrange a Medical Practitioner as applicable
 - Immediate medical care is the first priority for treatment of injuries. A secondary priority is to obtain forensic evidence. Ideally, the medical and forensic examination and treatment should be completed simultaneously, with patient consent
 - Forensic sampling is time sensitive and is to be conducted by SARC clinicians at the first possible opportunity/within 12 hours from initial presentation and gathering of evidence (but must not compromise urgent medical care). In some cases, forensic evidence may only be available for up to 24 hours after the assault. After 10 days, it is highly unlikely that forensic evidence can be collected. Refer to SARC Referral criteria section.
2. Treat any disclosures made as confidential
 - If the report is made via telephone (e.g., community patient), arrange for an appropriate clinician to meet with the patient face to face to discuss
 - Ensure a professional interpreter is present if required – family, friends or other non-qualified interpreters are not to be used
3. Establish a brief history of what has happened and when to determine an appropriate course of action (refer to SARC Triage, Checklist and Referral Form for guidance available at [SARC website](#)) Considerations:
 - Age -
 - Under 13 years contact Child Protection Unit (CPU) 08 6456 4300
 - 13 – 18 years contact SARC 1800 199 888
 - 18 years and over - consider contacting SARC 1800 199 888
 - Capacity – if diminished, escalate to consultant, consult with Social Work regarding reporting to the Police and respond appropriately
 - Communication skills/interpreter needs

- Non-urgent medical issues to include emergency contraception, testing for sexually transmitted diseases (STIs), minor injuries, pre-existing conditions, and pregnancy in consultation with SARC
 - Physical Examination Form to be completed and medical history obtained
 - Current mental state and risk e.g., anxiety, risk of self-injury, suicidality, psychosis, mania, risk to others, risk from others
 - Mental Health Clinician to complete RAMP if any concern about person's mental health. General Health areas to refer to Psychiatrist/Consultation- Liaison (C-L) Psychiatry Services
 - Any history of non-fatal attempted strangulation, previous trauma and/or assault
 - Involuntary status under the Mental Health Act (MHA) 2014
4. Provide psychosocial support and offer information and choices regarding processes, rights, and treatment options as appropriate
- Consider medical treatment and forensic examination
 - Consent is required for physical examination, genital examination, collection of medical specimens, collection of forensic specimens and reporting to the police where applicable
 - Provide information on refuge/emergency accommodation, domestic violence services, legal services, Centrelink, Crisis Care – contact Social Work Department for referral information
 - Refer to Social Work Dept. for all inpatients and consider referral to Social Work Outpatients where applicable
5. Ask the person if they wish to involve the police/encourage to report to Police
- Obtain consent if person has capacity to involve police and contact police immediately Sex Assault Squad (WA Police) 08 9428 1600
6. Consider referral to SARC if patient consents, see [SARC referral Criteria](#).
7. Report information gathered, and any actions taken to the patient's lead clinician/ Consultant and relevant nursing and medical leads and escalate as necessary
8. Request consent to inform patient's family/support person and assist where applicable
9. Ask the patient if they consent to forensic examination.
- If applicable arrange for SARC Doctor to complete forensic sampling and examination at the first possible opportunity/within 12 hours from initial presentation
 - Encourage the patient not to shower/wash/pass urine prior to examination so that a proper assessment can be made
 - If applicable, store separately in separate sealed, labelled paper bags all articles of clothing (if accessible) worn at the time of the incident. Refer to [AKG Sexual Assault Guideline](#).
 - Medical Practitioner to discuss medical history and physical assessment with SARC clinician

10. For those patients who have experienced a recent incident of sexual assault with identified high-risk indicators e.g., history of non-fatal attempted strangulation; history of previous trauma or assault; at risk of suicide, serious self-injury or causing injury to others:
 - Review/update the person's mental health risk assessment. Seek input from Consultant/ Psychiatry Team as needed
 - Escalate concerns relating to physical and psychological wellbeing by alerting medical staff or senior nursing staff immediately for prompt assessment
 - Provide further support as appropriate to a post critical incident e.g., referral to counselling services if appropriate
11. Consider the safety and wellbeing of other patients and staff and escalate/act accordingly
 - Consider the use of additional staff/increased observations/1:1 Nurse Specials/ transferring patients where applicable
12. Provide information regarding follow up available at a later date e.g., self-referral to SARC at any point, counselling, and medical follow up
13. Complete all mandatory reporting and OCP reporting requirements
14. Complete an alert notice in the integrated progress notes and Psychiatric Services Online Information System (PSOLIS/WebPSOLIS) as clinically applicable within 24 hours of the incident. Refer to [AKG Clinical Handover Procedure](#) to communicate risk alerts
15. Record in the integrated notes that an allegation of a sexual assault has been reported and any actions taken. **Do not record specific details of the alleged sexual assault in the consumer's health record.**

Documentation

Comprehensive and contemporaneous documentation pertaining to an alleged sexual assault is to be kept and maintained separately from the integrated notes as per the [WA Health Complaints Management Policy MP0130/20.](#), including the person's consent regarding reporting to police/SARC, advising family/support person, treatment, referrals made and plans for follow up care/other actions taken in a separate confidential file provided to the immediate line manager.

16. Ensure a Datix CIMS is completed within 24 hours of the incident

4.2 Considerations when an allegation is against another AKG patient.

1. Report to alleged perpetrator's lead clinician/Consultant and nursing and medical leads, and escalate as is necessary
2. Refer/arrange an assessment of the alleged perpetrator's current physical and emotional well-being and ensure support by an appropriate clinician
3. Provide psychosocial support and offer information regarding processes, rights, and process for reporting to Police
4. Store separately in sealed, labelled paper bags all articles of clothing (if accessible) worn by alleged perpetrator at the time of the incident (if applicable). Refer to [AKG Sexual Assault Guideline](#).
5. Consider the use of increased observations/ 1:1 Nurse Specials and transferring person to another unit/service/ward where applicable – ask person for preference where practicable

6. Documentation noted as above

4.3 Considerations when an allegation is against an AKG staff member.

1. Report immediately to line manager who will notify the site Executive and Human Resources Director and Integrity and Ethics in order that the relevant notifications can be made
2. Refer to **EMHS Inappropriate workplace behaviours; reporting of Guidelines and DoH Notifiable and Reportable Conduct Policy and DoH Discipline Policy and Guide**
3. Any medical care required is to be accessed from a service other than where the alleged incident occurred.

Armada Mental Health Service (AMHS) staff work within the boundaries prescribed by national, professional, legal and WA Health codes of conduct and practice. At no time is it acceptable for a staff member employed within the Armada Mental Health Service, to engage in sexual contact with patients/consumers, regardless of the relationship being initiated by the patient/consumer and whether or not it is considered consensual.

4.4 Considerations when an allegation is disclosed by an AKG staff member.

1. Provide psychosocial support
2. Notify staff member's line manager/after-hours with permission of staff member
 - Fitness for work assessment to be considered
3. Offer information regarding processes, rights, and process for reporting to Police
4. Support staff member to contact SARC and/or encourage GP/ED attendance as applicable

5. SARC referral Criteria

If the person consents to be referred to SARC, the criteria for referral includes¹:

- For a sexual assault which has occurred within the past 2 weeks (recent), staff/patients are to contact SARC 1800 199 888 for emergency/brief telephone counselling (available 0800 – 2300 hours, 7 days/week)/ medical and/or forensic examinations for persons over 13 years immediately or within 24 hours from when initially informed of the incident
 - If photography of any general body injuries is required, this can only be completed at SARC for Perth metropolitan areas
 - Medical staff are to speak to SARC medical staff (24-hour service) to confirm if the patient's sexual assault medical needs have been addressed, prior to the consumer being triaged by a SARC Counsellor
- If over 2 weeks since the sexual assault, or if incident/incidences are historical, refer the patient to SARC counsellors for triaging and for emergency counselling but forensic sampling is unlikely. Patient can self-refer. Please see
- If over 6 weeks since the sexual assault, patient/staff are to call for non-urgent counselling
- Advise the patient that the initial counselling/medical and/or forensic appointment can take 2-4 hours (with SARC Counsellor presence for support, if required)

- Offer the patient's family/support person referral for brief counselling services

Note: Evidence can be collected and stored for 6 months with SARC if the patient is unsure if they want to report to police. After 6 months, SARC will contact the patient to find out if they have made a decision about reporting the incident to police.

6. Compliance monitoring

Compliance against this procedure will be monitored by the Mental Health Safety and Quality Departmental Committee.

Compliance monitoring activities relevant to this procedure include:

- Compliance will be monitored as part of the Datix CIM process
- Combined Hazard or Incident Reporting (CHoIR)
- OCP Reporting Notifiable Incidents
- The annual State-wide Standardised Clinical Documentation (SSCD) audit
- The Compliments or Complaints Management System
- Human Resources' disciplinary reporting

7. Policy context

The following documents are required to give effect to this procedure:

- *Mental Health Act 2014*
- Health Department of Western Australia (HDWA) [Responding to an Allegation of Sexual Assault Disclosed Within a Public Mental Health Service](#)
- HDWA [Guidelines for Protecting Children 2015 OD 0606/15](#)
- HDWA [Clinical Incident Management Policy 2019 MP 0122/19](#)
- HDWA [Complaints Management Policy MP 0130/20](#)
- HDWA [Notifiable and Reportable Conduct Policy MP 0125/19](#)
- East Metropolitan Health Service (EMHS) [Mental Health Advocates EMHS: 162](#)
- EMHS Critical Incident Stress Debriefing Policy
- EMHS Acute Response Mental Health Emergencies /Crisis Policy
- EMHS Notifications Policy
- EMHS Consent Policy
- EMHS Employee Assistance Program Policy
- [AKG Sexual Assault Guideline](#)

8. Supporting information

The following documents support the implementation of this procedure:

- MR Form reference
- Patient information
- Education and training resources

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.01
- Standard 2 – Action 2.01

National Standards for Mental Health Services (2010):

- Standard 1 – Criteria 1.1

Australia/New Zealand Standards™ (AS/NZS):

- AS/NZS 0000:0000

10. References

[References are to include any texts or other site/organisation policies used to develop the policy. All references are to be entered in APA 6th ed. format. A free formatting guide can be found here at the University of Western Australia: <http://guides.is.uwa.edu.au/apa>]

1. Sexual Assault Referral Centre. (2017). *Procedure for Responding to a Recent Sexual Assault*. Department of Health. Western Australia.

11. Glossary

The following definitions are relevant to this procedure:

Term	Definition
Child	Anyone under 18 years of age, or in the absence of documented evidence, appears to be aged under 18 years.
Sexual Assault	Any unwanted sexual act or behaviour which is threatening, violent, forced, coercive, or exploitative and to which a person has not given or was not able to give consent.
Consensual Sexual Activity	Where all parties are: • Of legal age - 16 years or older in WA • Awake, conscious and not drugged or intoxicated • Developmentally and cognitively able to understand what activity takes place • Consenting to activity without threat, force, coercion, or intimidation.

12. Document owner

Karen Elliott – Service Director Mental Health

Enquiries relating to this procedure may be directed to:

Title: Dene Olive
 Department: Mental Health
 Email: dene.olive@health.wa.gov.au

13. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
AK0026/23	04/07/2023	04/07/2026	Minor Amendments

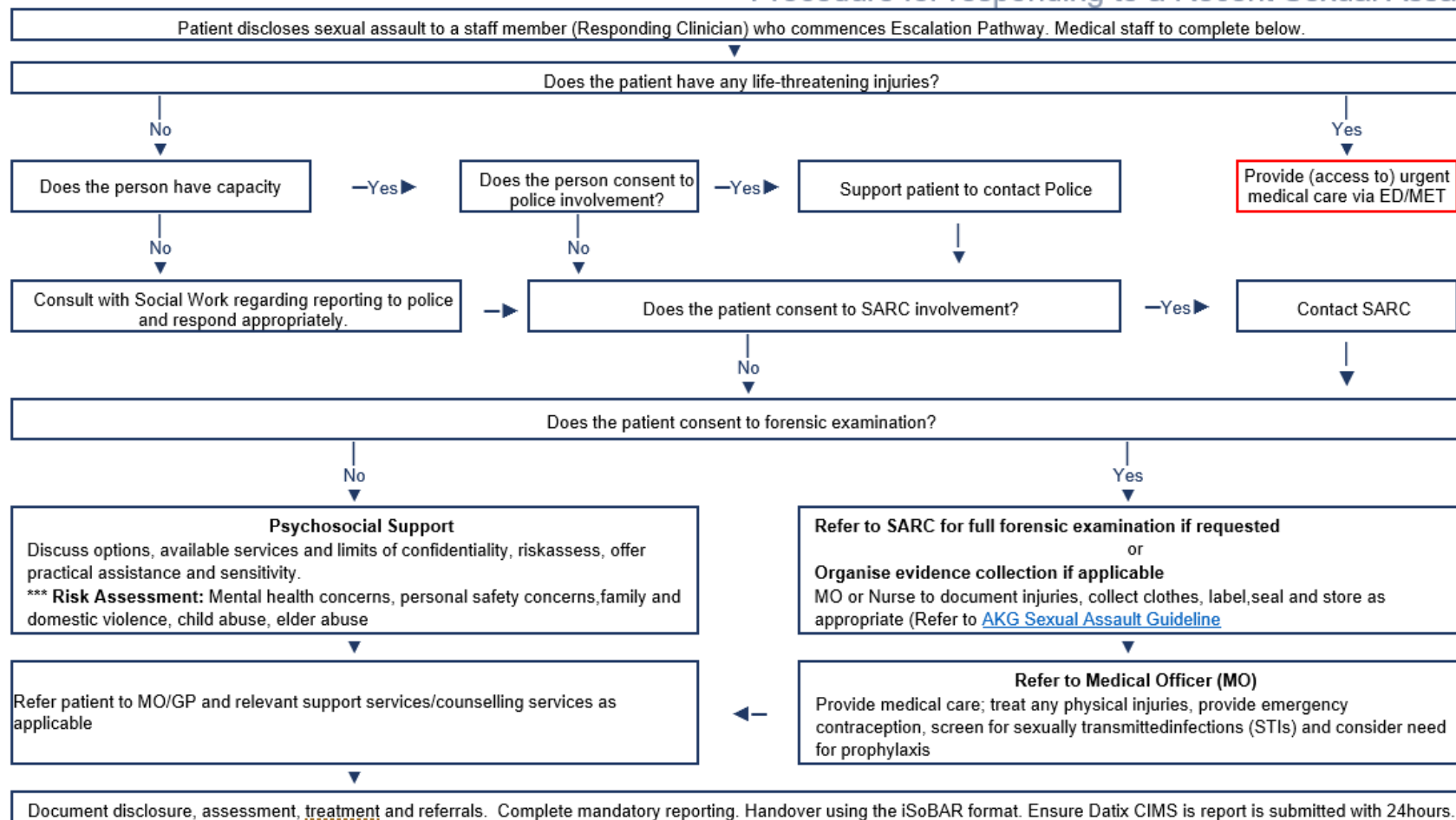
14. Authorisation

This procedure has been authorised and issued by:

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	04/07/2023
Published date	04/07/2023

Appendix 1

Procedure for responding to a Recent Sexual Assault



Adapted from the SARC Procedure for Responding to a Recent Sexual Assault²

Appendix 2

Escalation Pathway

Inpatient

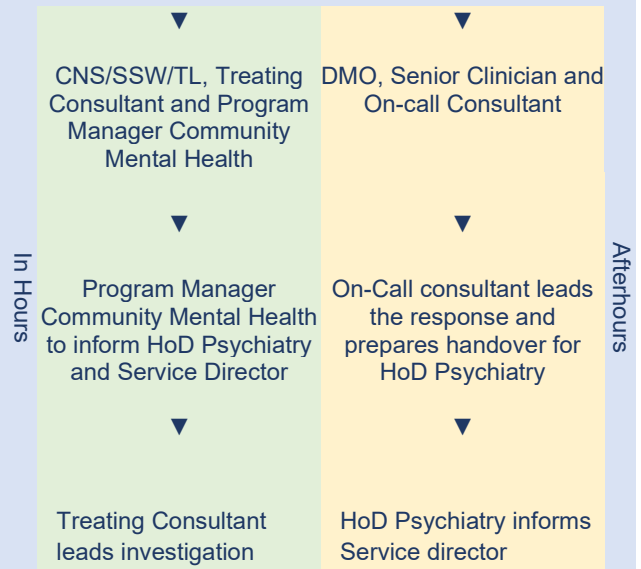
Shift Coordinator is immediately informed by Responding Clinician, enacts strategies to ensure patient safety and informs:



Briefing note is completed by consultant as above to report progress and status to the HoD of Psychiatry and Service Director

Outpatient

Care Coordinator is immediately informed by Responding Clinician, enacts strategies to ensure patient safety and informs:



Briefing note is completed by consultant as above to report progress and status to the HoD of Psychiatry and Service Director

Considerations to be present in report:

- Age
- Capacity
- Language skills
- Non-urgent medical needs
- Significant history
- Mental well-being
- Involuntary status under the Mental Health Act (2014)

Appendix 3

Responding to Allegations of sexual activity – Inpatient After hours checklist				
		Yes	No	N/A
AH CNS	Immediately report to Duty Medical Officer and On Call Consultant			
	Allocate staff member to stay with client initially, provide support and care			
	Bed Movement			
	Increased SASO			
	Organise prompt medical review			
	Encourage not to shower, bathe or wash prior to examination by Sexual Assault Resource Centre staff, as vital forensic material/evidence may be lost.			
	CIMS completed			
	Inform the NOK/PSP if client has given consent			
	Ensure clear communication is maintained to the client to ensure they are aware steps are being taken to maintain their safety			
On-Call Consultant	The On-Call Consultant Psychiatrist will make an immediate assessment of the report and resolve what actions should be taken. Actions may include: - Referral of the complaint to WA Police and/or SARC. Expected Timeframe of no greater than 1 hour from initial complaint to make this decision. <i>Sex Assault Squad (WA Police) 08 9428 1600</i> <i>SARC 1800 199 888</i> - Continuation of the investigation of the alleged incident. - Determining the need to transfer the patient to another service or inpatient area and discussing the option with the patient.			
	On-Call consultant documents the reasons for decision taken			
	The On-Call Consultant Psychiatrist will provide a briefing note to the HoD Psychiatry as soon as practicable			
Documentation pertaining to an alleged sexual assault or reported sexual contact should be kept and maintained separately from the consumer's medical record as per the WA Health Complaints Management Policy MP0130/20 .				